

Case Discussion

IT'S A PAIN

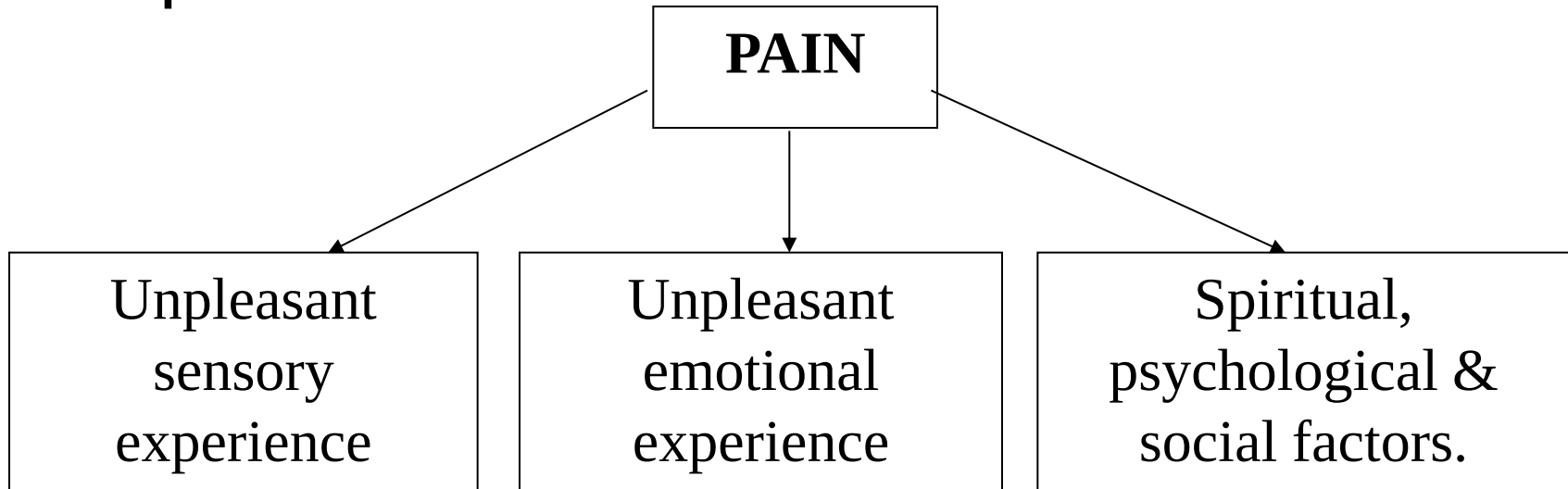
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Setting the Scene



Classification

Pain is traditionally classified into different modalities: physical, psychosocial & spiritual.



- Age - 38
- Sex - female
- Marital status - Married
- Dependants - 2 Children 5 & 7
- Absence duration - 7 months
- Incapacity – Neck and lower back pain
with right arm pain
- Occupation - PA to MD
- Right hand dominant

Pertinent Medical information

- Initially treated by GP, with an anti-inflammatory and was referred to Orthopaedic, following failure to improve.
- Orthopaedic arranged for X-ray's and MRI scan.X-ray's showed normal findings
- MRI scan showed slight degeneration of C spine & lumbar spine with no nerve compression.
- Orthopaedic discharged and referred to Physiotherapy to improve core stability and posture.
- GP prescribed Co-Codamol 30/500mg 2x qds
- Attends physio once due to increased pain and unable to perform any exercises due to reported pain. Physio referred back for further orthopaedic opinion.
- Orthopaedic noted severe pain on rotation and flexion of neck with radiation into right arm. Restricted flexion in lumbar spine with significant pain. SLR's normal on both sides. No red flags. Refers for further MRI. Result of this is unchanged from previous MRI. Refers on to Pain Clinic.

- Continues to report significant pain to the GP and he changes her analgesia to Tramadol 50mg qds, and Amitriptyline 10mg nocte.
- Attends Pain Clinic assessment - recommendations:
 - CBT for psychological management.
 - Increase of Tramadol to 100mgs 4 qds and Amitriptyline 20 mgs nocte.
 - Daily exercises given.
 - Review in 3 months
- Note: CBT waiting list is 3 - 4 months.

WHO Analgesic Ladder 1998

Non- opioid
(Paracetamol) +/-
adjuvant/ co-analgesic
(Diclofenac)

STEP 1

Weak opioid
(Co Codamol)
+ non-opioid
+/- adjuvant/
co-analgesic

STEP 2

Strong opioid
(Tramadol 100mg
qds)
+ non opioid
+/- adjuvant/ co-
analgesic
(Amitriptyline)

STEP 3

Reported Restrictions & Limitations

- Unable to sit for longer than 20 minutes
- Unable to carry heavy shopping
- Unable to type for longer than 10 minutes
- Unable to use mouse for more than 10 minutes
- Unable to bend to empty washing machine
- Unable to stand for longer than 10 minutes.
- Unable to get in/out of the bath
- Cannot walk long distances
- Cannot drive long distances
- Pain on moving neck



Activities of Daily Living

- Get up at 7 am, shower & dress
- Prepare breakfast for children, & wash up
- Assist children getting ready for school. Drive children to school
- Go to local shops
- Arrive home, make a drink and rest for 15 minutes.
- Do some light houseworkslowly.
- Pop to friends for coffee and lunch
- Walk the dog to the local park
- Collect children from school
- Prepare dinner & wash up.
- Help children with homework
- Put children to bed
- Watch TV
- Go to knit and natter once a week.
- May go out for dinner with friends at the weekend.



What Next?



Questions about the workplace

- What are the daily duties of their occupation?
- Did they enjoy work?
- Did they have good relationships with colleagues/bosses?
- Do they still keep in contact with manager/colleague?
- Did they have childcare arrangements, are these still in place?
- Has their condition improved away from the workplace?
- What are their barriers to returning to work?
- What do they think could help them return to work?





Questions on ADL's

- Tell me about your typical day now?
- How far is it to your children's school?
- How do you find getting in and out of the car?
- Do you ever walk to school?
- Do you walk the dog to the park every day? Do you find walking helps? How far is it to the park?
- Tell me about your condition, was there any particular cause?
- I see you advised you are limited to sitting for 10 minutes, has that improved at all? What kind of chair do you sit in at home?
- What helps to ease your pain?
- How long does your pain relief last after taking your medication?
- Do you need help with personal hygiene and care?
- Do you have home help/assistance?



Direct contact responses

- Used to love her job
- Job became more pressurised for a few months prior to absence.
- Needed to work longer hours in the last 3 months.
- Not had much contact with work during her absence.
- Mother-in-law would take and collect the children from school. Still does this if the claimant is having a bad day.
- Thinks longer hours and work pressure caused symptoms to worsen. Always had some neck, back and arm pain.
- Barriers to working; in too much pain, could not type or use mouse for longer than 10 minutes. Could not concentrate due to pain. Unable to sit for longer than 10 minutes in chair. GP has told her that she should not go back to work until she feels better.
- Main duties, taking telephone calls, booking meetings, arranging diary for MD, putting together presentation packs, highlighting urgent emails, taking minutes. Making tea/coffee. Over 90% of day at desk.

- Typical routine as previously described.
- Drives children to school which is 10 minutes away by car.
- Has 4 wheel drive so able to get in/out without problems.
- Would not be able to walk the 20 minutes to school due to pain.
- Walks to park 10 minutes and can sit on bench to rest. Walking does help sometimes.
- Sits on dining room chair to use laptop, read paper and eat meals. Has a cushion for her back.
- Is able to sit on the sofa, but finds getting up difficult. Can sit on the sofa for longer than the dining room chair. Approximately 20 minutes rather than 10.
- Can do all housework slowly and carefully, but finds getting washing in and out difficult and painful. It affects her neck and back.
- No problems with personal care. Showers rather than bathes.
- Pain relief lasts 3-4 hours, but feels groggy and there is always a dull pain which is exacerbated on too much activity.
- No home help. Goes supermarket shopping with her husband.

Lets Discuss the Evidence





The Evidence

- Are the claimant's current symptoms objective or subjective?
- What are the functional demands of her insured occupation?
- Has the current treatment improved the reported symptoms?
- Do the claimant's reported activities of daily living match her reported restrictions?
- Is the function required to perform the reported activities of daily living comparable with the function required for the performance of the occupational demands?
- Is the medical evidence consistent with the claimant's reporting?
- Does the frequency of prescriptions indicate that medication is being taken on a regular basis?
- Can the claimant's barriers to returning to work be overcome?
- Are there any non medical barriers to a return to work?

What Next?





Other Tools

- Surveillance
- Functional Capacity Assessments.
- IME
- Report from treating consultant
- Home visit
- Rehab visit
- Paying for CBT

THANK YOU FOR
YOUR INPUT.